NCCP 00776 · clinician draft

Trastuzumab deruxtecan (Enhertu) · every 3 weeks

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · Trastuzumab deruxtecan (Enhertu) · every 3 weeks

Active medicines: Trastuzumab deruxtecan (Enhertu)

Emetogenic risk: High · dedicated trastuzumab deruxtecan row

Medicine	Dose / route	When
Aprepitant	125 mg orally	Day 1 before treatment
Ondansetron	16 mg orally	Day 1 before treatment Review QT risk, hepatic impairment, constipation and interacting medicines.
Dexamethasone	12 mg orally	Day 1 before treatment Reconcile all steroid premedication; do not duplicate.
Aprepitant	80 mg orally once daily	Days 2–3
Dexamethasone	8 mg orally once daily	Days 2–4 Confirm suitability and total steroid exposure.
Metoclopramide	10 mg orally up to three times daily	Rescue use / duration to be completed by prescriber Review renal function and neurological contraindications; avoid automatic prolonged use.

Additional phase-specific support

- This regimen has high emetic risk. An antiemetic combination is prescribed before every infusion, with an agreed plan for delayed symptoms and breakthrough nausea. The dedicated NCCP trastuzumab-deruxtecan pathway should be checked by the prescriber and pharmacist.
- Carry the patient alert card and report new cough, fever or breathlessness promptly. Steroids used for suspected lung inflammation are prescribed and monitored by the clinical team; do not start them yourself from this information.
- Low blood counts can cause infection, anaemia or bleeding. The need for G-CSF or transfusion support depends on results, symptoms and previous complications; it is not automatic for every cycle.
- Small frequent meals and early treatment of nausea can help maintain intake. Tell the team about weight loss, constipation or diarrhoea so that support can be tailored.
- NCCP NCIS v8 Table1 has a dedicated trastuzumab-deruxtecan row: NK1 + 5-HT3 + dexamethasone. Olanzapine can be considered by the prescriber, but is not silently added to that national default row.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00776 v3b · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-20 · SACTCheck v0.84.9 · Regimen-level drafts; prescriber and oncology-pharmacy verification required